

Chronic Kidney Disease (CKD) in Primary Care

CKD Early Identification Pathway

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Optimising Identification & Management of Chronic Kidney Disease: A Rationale

What is Chronic Kidney Disease?

CKD is “a reduction in kidney function or damage to structure over a period of 3 months with associated health implications.”

Albuminuria is the most common marker of kidney damage.

A high-risk condition for cardiovascular disease.
£1.45 billion in annual costs to NHS.

Failure to identify and treat CKD doubles mortality, meaning it is important to ensure people are on optimal therapies **as soon as possible**.

Why does CKD matter?

What does the evidence tell us?

Albuminuria is an independent risk factor for progression to end stage kidney disease and cardiovascular mortality, at any eGFR. uACR is therefore **essential** in combination with eGFR to diagnose CKD in high-risk patients.

Diagnosing and coding CKD **early** enables people to access interventions such as lifestyle advice and pharmacotherapy to reduce the risk of CKD progressing and of significant cardiovascular complications.

- ✓ RAS/RAAS blockade is recommended by NICE (CG203, 2021) for those with an uACR >70mg/mmol, or with an uACR >30mg/mmol if hypertensive.
- ✓ Significant clinical benefit demonstrated with combined RAS/RAAS blockade and SGLT-2 inhibition, even at uACR as low as 22.6mg/mmol in people living with CKD (1, 2).
- ✓ An SGLT-2 inhibitor in combination with maximum tolerated RAS/RAAS blockade therapy is **therefore now recommended** by NICE (TA775, March 2022) for:
Adults living with Type 2 diabetes and chronic kidney disease *and*
Adults living with chronic kidney disease without diabetes, if their uACR is >22.6mg/mmol.

London Kidney Network: The challenge for London & Surrey Heartlands



- High levels of social deprivation
- Ethnically diverse populations
- Poor outcomes due to high rates of undiagnosed and uncoded CKD
- Significant variation in clinical practice

The London Kidney Network's vision is to support primary care to identify CKD **early**, reduce variability in detection and management, and optimise interventions such as RAS/RAAS blockade and SGLT-2 inhibitors to **save lives**.

The **Kidney Health Check** for Adults Living with Diabetes or Hypertension:

How to identify Chronic Kidney Disease *early!*

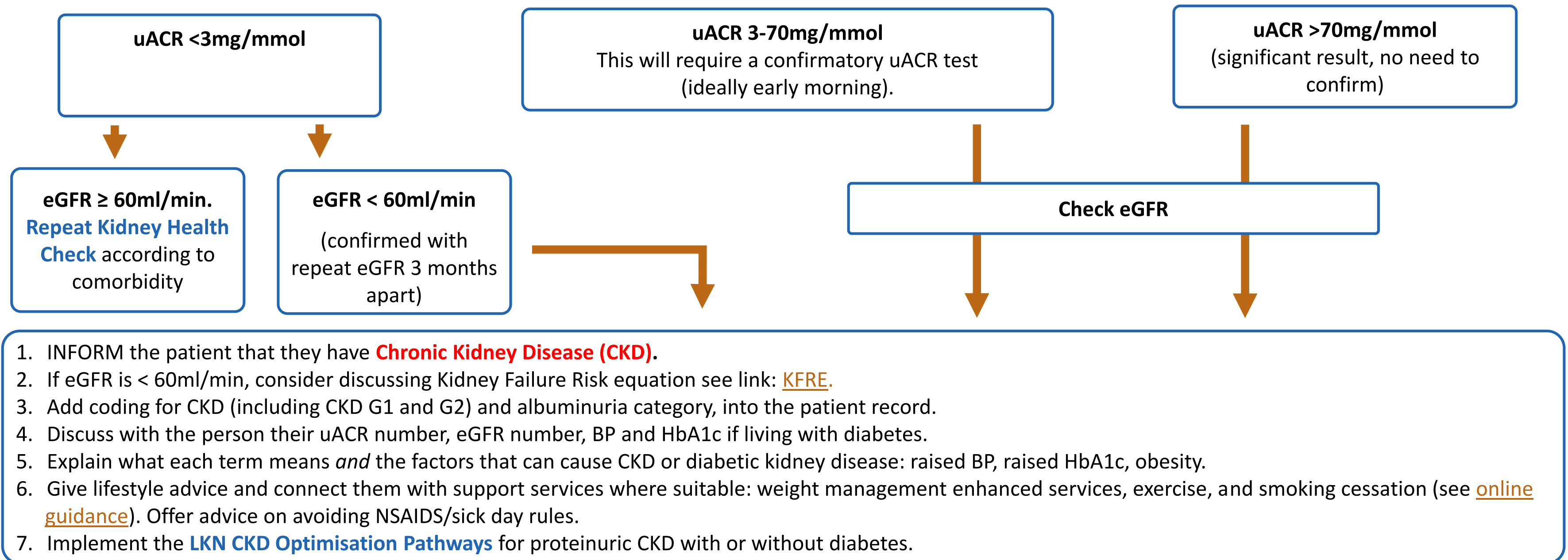
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What is a **Kidney Health Check?** It is the combination of both an **eGFR** *and* a **uACR** test

Who should have a **Kidney Health Check?**

1. People living with **diabetes** should have a yearly kidney health check
2. People living with **hypertension** should have a kidney health check every 1-5 years (annually for poorly controlled hypertension)
3. See [NICE CKD Assessment and Management](#) for ACR testing in other health conditions



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References & Acknowledgements

References

1. Heerspink HJL, Stefánsson, BV, Correa-Rotter R, et al. Dapagliflozin in patients with chronic kidney disease. *N Engl J Med*. (2020) 383:1436-1446. doi:10.1056/NEJMoa2024816
2. Perkovic V, Jardine MJ, Neal B, Bompoint S, Heerspink HJL, Charytan DM, et al. Canagliflozin and renal outcomes in type 2 diabetes and nephropathy. *New Engl J Med*. (2019) 380:2295–306. doi: 10.1056/NEJMoa1811744

The London Kidney Network reviewed the following guidelines in producing these pathways:

1. [Dapagliflozin for treating chronic kidney disease \(NICE TA775, published March 2022\)](#)
2. [Chronic Kidney Disease: Assessment and Management \(NICE guideline NG203, updated November 2021\)](#)
3. [UK Kidney Association Clinical Practice Guideline: Sodium-Glucose Co-Transporter-2 \(SGLT-2\) Inhibition in Adults with Kidney Disease \(published October 2021\)](#)
4. [Clinical Practice Guidelines for management of hypertension and renin-angiotensin-aldosterone system blockade in adults with diabetic kidney disease: 2021 update \(UK Kidney Association and Association of British Clinical Diabetologists\)](#)
5. [Hypertension in adults: diagnosis and management \(NICE guideline NG136, updated March 2022\)](#)

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